

tronic systems for hospital records. Take, for instance, the Shalby Hospital, a 240-bed multispecialty private hospital in Ahmedabad. It transitioned from a small 15-bed single specialty unit established in 1993 to a technologically-advanced 200-bed multispecialty hospital in 2007. The hospital has invested heavily in IT infrastructure with Cisco and Nortel networking infrastructure. It has a fully integrated hospital management and information system, designed to manage every aspect of information flow and control across the hospital—right from vendor records to patient data—electronically. "Our goal is a paperless office," says Dr Vikram Shah, managing director and an orthopaedic surgeon. Shalby is also one of the first

hospitals in the country to pioneer incorporation of infection control measures like HEPA Filters, Laminar Air Flow, Body Exhaust System, Plasma Sterilizer and Maquet Operating Tables.

An unintended consequence of the flourishing healthcare market has been a nation-wide "poaching" of human resources. "There's a lot of poaching going on," endorses Dr Ashok Seth, head of Escorts Heart Institute & Research Center, Delhi. Not only are people moving out of government hospitals, corporates are taking from each other too. "But these are market forces that you have no control over. You cannot make legislation to stop this. It's a free country, after all!" The prime driver is the compensation packages that corporate hospitals of-

fer, often five to 10-times more than the market rate. "Some have 20 different scales and salary systems—with fixed and variable pay," he says. But not everyone would be eligible for the top slots. Even a reputed doctor would have to conform to various market-driven parameters to justify such a jump. A doctor's field, his reputation capital and his finance-generating capacity will have to tally with the 'needs' of the healthcare corporate. "Someone in cardiac, for instance, will be able to dictate terms more than a skin specialist," he adds. "They consider the returns that we will get by investing on a certain person." The bottomline in salary negotiation, however, is that the patient is king. "A patient doesn't care how much you

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